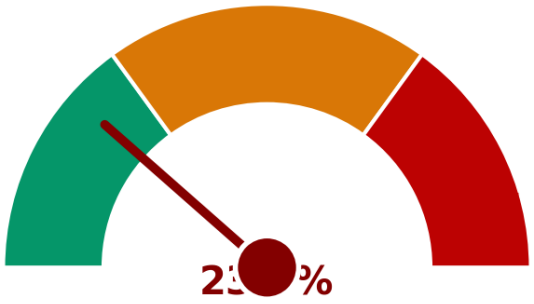
CardioAI Diagnostics

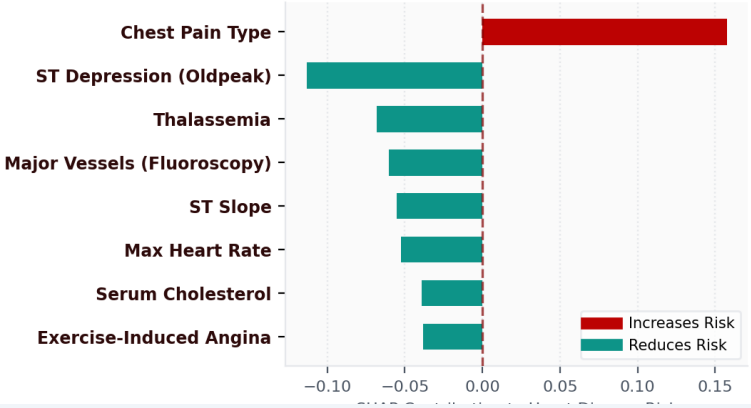
Clinical Cardiovascular Risk Assessment

Report ID: CAI-20261013-74EEFB
Date: October 13, 2026 - 14:37 UTC

Patient Name: Patient

* Age: 66 years	* Sex: Male
* Chest Pain Type: Asymptomatic (3)	* Resting Blood Pressure: 170 mmHg
* Serum Cholesterol: 320 mg/dl	* Fasting Blood Sugar: Elevated (> 120 mg/dl)
* Resting ECG: Left Ventricular Hypertrophy (2)	* Max Heart Rate Achieved: 98 bpm
* Exercise Induced Angina: Yes (Induced)	* ST Depression (Oldpeak): 3.5
* Peak ST Slope: Flat (1)	* Major Vessels (Fluoroscopy): 3 major vessel(s)
* Thalassemia Scan: Reversible Defect (3)	





AI RISK ASSESSMENT SUMMARY

Classification: LOW RISK (Probability: 23.0%)

Personalized Clinical Recommendations:

- MAINTAIN: Continue routine preventative cardiovascular health checkups annually.

- High Cholesterol Alert (320 mg/dl): Exceeds recommended 200 mg/dl threshold. Evaluate lipid profile and adopt a low saturated fat, Mediterranean diet.

- Hypertension Flag (170 mmHg): Resting systolic pressure indicates Stage 2 hypertension. Daily blood pressure logging and sodium reduction (< 2,000 mg/day) strongly advised.

- Exercise Angina Detected: Patient reports exercise-induced chest pain. Avoid unmonitored strenuous aerobic exertion until cardiology clearance.

- Marked ST Depression (3.5): Electrocardiographic ST depression during stress suggests significant myocardial ischemia.

- Elevated Fasting Glucose (> 120 mg/dl): Potential glycemic dysregulation. Check HbA1c to screen for insulin resistance, which accelerates arterial plaque.

- Coronary Calcification Marker: 3 major vessel(s) detected with fluoroscopy calcification, an established marker for coronary artery disease.

DISCLAIMER: CardioAI is an educational screening and risk estimation tool, not a certified clinical diagnosis. Decisions regarding patient therapy or lifestyle